

Rare Conditions Imitating Rosacea

Rare conditions that may mimic rosacea include **Morbihan's disease**, which causes persistent lymphoedema and erythema affecting the central and upper regions of the face. It can be clinically similar to **Melkersson-Rosenthal syndrome**, which also features solid, persistent facial oedema along with recurrent facial nerve palsy and fissured tongue.

Rosacea fulminans, formerly known as pyoderma faciale or rosacea conglobata, occurs almost exclusively in young women, typically in their 20s and 30s. It presents suddenly with intense facial inflammation, including erythema, papules, sterile pustules, coalescing cystic nodules, and draining sinuses. Key differential diagnoses include **acne conglobata**, **Gram-negative folliculitis**, and **lupus erythematosus**. A **Gram-negative variant of rosacea** has also been reported.

Steroid-induced rosacea is an under-recognized but important condition, resulting from prolonged systemic or topical glucocorticosteroid use. It initially manifests as macular erythema with telangiectasia, progressing later to follicular papules and pustules. Discontinuation of steroids is essential, although this may temporarily worsen lesions.

Granulomatous rosacea (also called lupoid rosacea) is characterized by reddish-brown papules or nodules appearing alongside diffuse facial erythema. Atypical presentations of rosacea may involve persistent erythema in unusual sites such as the scalp or scrotum.

Differential Diagnosis Process

The clinical diagnosis of rosacea relies on patient history, symptoms, skin findings, age, and gender. Typical features include **centrofacial erythema**, **telangiectasia**, **papules and pustules**, and **phymatous changes** involving the nose, chin, glabella, and forehead.

Features less consistent with rosacea should be evaluated to rule out other conditions:

- Extensive scaling (suggestive of seborrhoeic dermatitis or tinea)
- Sharply demarcated erythema (as seen in erysipelas)
- Widespread scarring
- Unilateral inflammation (seen in demodicosis)
- Perioral or periorbital inflammation with a narrow non-inflamed zone around orifices (typical of perioral dermatitis)
- Marked photosensitivity (seen in lupus erythematosus or polymorphic light eruption)

In most cases, diagnosis remains clinical, and skin biopsies are not routinely required. However, it is critical not to overlook **angiosarcoma**, which can resemble rosacea in its early stages.

Biopsy with **direct immunofluorescence** is recommended when lupus erythematosus is suspected; comparison with a sample from non-sun-exposed skin helps prevent false-positive results.

Clinicians should consider differential diagnoses such as **lupus erythematosus**, **polymorphic light eruption**, **perioral dermatitis**, **acne**, and the increasingly recognized **mixed facial dermatosis**, which exhibits overlapping features of **seborrhoeic dermatitis** and **rosacea**.

Dermatopathology (Skin Biopsy)

Histopathologically, all subtypes of rosacea show:

- Dilated blood and lymphatic vessels in the upper and mid-dermis
- Superficial perivascular

and perifollicular mononuclear (lympho-histiocytic) infiltrate -

Possible dermal oedema and thickened elastic fibres

In subtype I (erythematotelangiectatic rosacea), histological changes are often minimal.